data from Phase I and Phase II trials and was asked in the same meeting to submit safety and efficacy data from the ongoing Phase III trials in the country in order to be further considered for the approval.[22]

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While India's active caseload was consistently on the decline since its peak in September, several countries of the world were being hit by a second wave of infections. The UK was particularly seething from the rapid surge of the second wave seemingly brought upon by a new strain of the coronavirus—Alpha variant (B.1.1.7), which was more transmissible and fatal, compelling the UK government to announce a slew of new sweeping restrictions.[23] As mentioned earlier, several positive samples from Delhi, Punjab and Haryana at the time were attributable to this variant.[24]

To keep a close watch on this and other probable variants and mutations, India set up a network of 10 government laboratories called the Indian SARS-CoV-2 Consortium on Genomics (INSACOG). Private laboratories, although capable, were disallowed to carry out genome sequences on samples in order to ensure that a certain testing standard was maintained on sequencing. There was also a stringent protocol which needed to be followed for sample collection and analysis in order to put the data in a set which would make sense epidemiologically. Let me elaborate, as of October 2021, while India had the capacity to sequence over 1,00,000 samples, the challenge was not the capacity to test, but rather procedural. Sequencing is not just sequencing; connecting the dots so that data can be analysed to make contextual sense is more important. In order to carry out genome sequencing effectively, end-to-end tracking and follow-up with each patient giving their sample is required. In India's case, capacity restrictions are not the bottleneck. Getting samples which are tagged with correct metadata including details and clinical symptoms of the associated individuals is a mammoth task in a country like ours, where the system is already fatigued and overwhelmed.

It is INSACOG's efforts in December 2020 which yielded conclusions that several positive samples from north India were caused due to B.1.1.7 strain (Alpha). A cause for worry.